



Endoscopy Nursing Care Procedure

1. Scope

Site	Operational Area	Applicable to
Armada Kalamunda Group	Same Day Unit, Pre-Admission Clinic and Day Procedure Unit	Nursing

2. Purpose

To outline the minimum pre and post procedure nursing practice standards required for colonoscopy, flexible sigmoidoscopy, and gastroscopy.

3. Procedure

3.1 Prior to admission

Patient procedural information should be provided prior to the endoscopy date by nursing staff from the Pre-Admission Clinic or inpatient ward, and the relevant bowel preparation completed prior to admission to the Same Day Unit (SDU) or Day Procedure Unit (DPU).

A Pre-Admission Clinic interview or telephone appointment should be completed for all outpatients prior to the patient's admission date, for assessment and education regarding fasting and bowel preparation.

If any issues are identified for patients booked at Kalamunda Hospital that may make them potentially unsuitable for treatment in the DPU the nurse should discuss these issues with the Endoscopy Lead Anaesthetist.

3.2 Admission to the Same Day Unit / Day Procedure Unit

- Verify the patient's identity and confirm that it matches the medical file and identity bands. Attach two identity bands to separate limbs ensuring that the colour of the band matches the patient's allergy/adverse drug reaction status.
- Orientate the patient to unit environment, identifying their nurse call bell and the nearest toilets.
- Complete all relevant sections of the AKMR 93.1 Endoscopy Care Plan and Record, ensuring that a set of pre-procedure observations are documented.
- Confirm that bowel preparation has been completed as instructed and that there is clear return on evacuation of the bowel.
- Ensure the patient has arranged for a responsible adult to transport them home and remain with them overnight (not applicable to inpatients).

- Instruct the patient to change into a theatre gown unless just having a gastroscopy and to visit the toilet prior to the procedure. Patients are allowed to wear small items of jewellery. Blankets should be available for warmth should the patient require one.
- Advise the patient to remain seated in the chair while they wait for their procedure unless there is a clinical need to remain in bed.
- Check that the completed consent form is in the medical file and that both the documented and proposed procedures are the same.
- Once checked-in by the Endoscopy staff in the SDU/DPU, the patient will walk to theatre escorted by an Anaesthetic Technician or Endoscopy Nurse (or be taken in a bed if required).

3.3 Management of Diabetic Patients

- For **all diabetics on regular SGLTIs** presenting for surgery, blood glucose and blood ketone levels should be routinely checked on admission.
- If the blood ketone level is >1.0 mmol/L, the case should be escalated to the anaesthetist in the theatre. If the anaesthetist feels the patient is unwell, an URGENT ABG or VBG should be performed to measure the Base Excess. (Please note - **In diabetics on SGLT2is**, if the person with diabetes has ceased the SGLT2i 3 days pre-procedure, is clinically well and ketones are < 1.7 mmol/L, it is safe to proceed.)
- A person with diabetes on SGLT2i undergoing **emergency surgery** should be admitted post-procedure to a ward capable of managing diabetic ketoacidosis in collaboration with endocrinology and critical care.
- **In diabetics on SGLT2is, if SGLT2 inhibitors have NOT been ceased 3 days** prior to surgery, the following process should be followed:

Ketones	Base excess	Comments
<1	> -5	No ketosis and no metabolic acidosis. Consider proceeding with day surgery: hourly monitoring of blood ketones during the procedure, and 2nd hourly following the procedure until eating and drinking normally or discharged. Where blood gas analysis is not available proceed only if added risk is consistent with goals of care. More extensive surgery: consider goals of care and collaboration with endocrinology and critical care. Perioperative insulin and glucose infusion may reduce risk.
>1	> -5	Ketosis without metabolic acidosis. Seek endocrinology or general medicine advice. Ketosis without acidosis may reflect starvation, particularly in individuals with HbA1c $< 9\%$ (<75 mmol/mol). Consider proceeding, but with perioperative insulin and glucose infusion to reduce risk of ketoacidosis.
>1	< -5	Ketosis with metabolic acidosis.

		Postpone non-urgent surgery. Escalate care with endocrinology and critical care. Urgent surgery to proceed with insulin and glucose infusion and ketone monitoring with guidance from endocrinology and/or critical care. (This situation is presumed to be euglycaemic acidosis if BSL<14 mmol/l, and DKA if BSL is higher, and needs escalation to endocrinology/medicine/critical care support).
Note 1: Blood gas analysis is recommended to assess for presence of metabolic acidosis. Where blood gas analysis is not readily available, and the ketones are > 1.0 mmol/L the procedure should not be performed. Note 2: HbA1c >9% or 75 mmol/mol is an indicator of insulin insufficiency. It confers a higher risk of DKA in this setting.		

Post-procedure Management

- Restart SGLT2i post-operatively when the person with diabetes is eating and drinking normally or close to discharge from hospital.
- A person with diabetes who has day surgery/procedures should only recommence SGLT2i when they resume full oral intake. Consider delaying commencement of SGLT2i for a further 24 hours but also consider potential for hyperglycaemia.

Provide the person with diabetes with written advice to seek medical advice if unwell in the week following the procedure.

4. Bowel preparation

- **Colonoscopy:** Bowel preparation should be completed prior to admission to the SDU/DPU as per the instructions provided. The admitting nurse should determine if bowel preparation has resulted in a clear return. If it is not clear, a Registered Nurse or Enrolled Nurse should administer a fleet enema unless contraindicated (see below). This does not require a prescription or verbal order.
- **Flexible Sigmoidoscopy:** A Registered Nurse or Enrolled Nurse should administer a fleet enema unless contraindicated (see below). This does not require a prescription or verbal order.
- **Gastroscopy:** Bowel preparation not required however nursing staff should check that the patient is adequately fasted.

The use of Fleet enemas is contraindicated for patients with:

- Megacolon
- Suspected acute severe colitis
- Bowel perforation
- Congestive cardiac failure
- Hypersensitivity to benzalkonium chloride or other quaternary ammonium compounds.

Medical advice should be sought in the following circumstances requiring caution:

- Impaired renal function
- Pre-existing heart disease
- Electrolyte imbalance (secondary to dehydration, calcium channel blockers or diuretics for example)
- Presence of a colostomy.

5. Post-procedure outpatient

- Recording of standard observations should be performed every 10 minutes and as clinically indicated for a minimum of 30 minutes from when sedation was last administered and include oxygen saturation, blood pressure, respiratory rate, heart rate and conscious level score.
- If moderate or severe bleeding per rectum is noted, the Endoscopist should be informed immediately.
- If the patient reports moderate to severe pain, this should be reported to the Endoscopist and Anaesthetist. The patient should be kept nil by mouth until reviewed.
- With the exception of patients who have had a gastroscopy with anaesthetic spray or gargle, provide diet and fluids once the patient is alert and sat in a chair (unless they are unable to transfer).
- For patients who have had a gastroscopy with anaesthetic spray or gargle, they should remain nil by mouth for 1 hour after the application of the local anaesthetic. After 1 hour, a drinking trial should be completed with a cup of cold water.
- If the patient is able to safely swallow the water, they can be offered food and a hot drink. If they are unable to safely swallow the water, seek advice from the Anaesthetist. Some patients may be required to remain nil by mouth for longer periods which will be specified in the post-procedure instructions.
- Remove the patient's intravenous cannula only when any pain and nausea are well controlled.
- Once a score of 9 or 10 is achieved, and the patient can tolerate diet and fluids, they may be discharged with transport provided by a responsible adult, unless otherwise specified in the post-procedure instructions.
- Prior to discharge give the patient a copy of their endoscopy procedure report and an Endoscopy Discharge Information sheet. The discharge checklist in the AKMR 93.1 Endoscopy Nursing Care Plan and Record should be completed.
- Provide the patient a copy of an attendance letter if requested.

6. Post-procedure inpatient

As above except:

- Calculate their score as per the normal post procedure process but exclude the “Ambulation” section. Once a score of 7 or 8 is achieved they may be transferred back to their inpatient ward, unless otherwise specified in the post-procedure instructions.
- Patients must stay a minimum of ten minutes in the SDU/DPU before transfer back to the inpatient ward.
- Place a copy of the endoscopy procedure report in the patient’s medical file prior to transfer.
- When back on the inpatient ward and with the exception of patients who have had a gastroscopy with anaesthetic spray or gargle, provide diet and fluids once the patient is alert.
- For patients who have had a gastroscopy with anaesthetic spray or gargle, they should remain nil by mouth for 1 hour after application of the local anaesthetic. After 1 hour, a drinking trial should be completed with a cup of cold water.
- If the patient is able to safely swallow the water they can be offered food and a hot drink. If they are unable to safely swallow the water, seek advice from the treating doctors. Some patients may be required to remain nil by mouth for longer periods which will be specified in the post-procedure instructions.
- If the patient has known dysphagia or difficulties with eating and drinking, the water drinking trial should not be attempted, and advice should be sought from the treating Speech Pathologist.

7. Documentation

- Nursing staff should follow the post-procedure instructions of the Endoscopist/Anaesthetist as documented in the patient’s notes or procedure report.
- Any issues or variances with the nursing care provided should be documented in the nursing progress notes within the AKMR 93.1 Endoscopy Nursing Care Plan and Record or on an MR55A Integrated Progress Note.

8. Legislative context

The following documents are required to give effect to this procedure:

- Health Department of Western Australia (HDWA) [Consent to Treatment Policy OD 0657/16](#)
- HDWA [Recognising and Responding to Acute Deterioration Policy MP 0086/18](#)
- East Metropolitan Health Service (EMHS) [Patient Identification and Procedure Matching EMHS: 21](#)
- EMHS [Discharge Community \(Written\) Policy EMHS: 12](#)

9. Supporting information

The following documents support the implementation of this procedure:

- [Documentation Standards – Clinical Procedure AKG-PRO-0134-11](#)
- [Clinical Handover Procedure AKG-PRO-0521-19](#)
- [Recognition and Response to Acute Deterioration Policy AKG-POL-0388-14](#)
- [Procedural Sedation Procedure](#)

10. Compliance, monitoring and evaluation

Compliance with this policy will be monitored via:

- Monitoring of reported clinical incident data via Datix CIMS.

Specific findings are to be reported at the relevant departmental meeting for review and identification of any opportunities for improvement.

11. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 2 – Action 2.3 Healthcare rights and informed consent
- The health service organisation has a charter of rights that is:
 - a. Consistent with the Australian Charter of Healthcare Rights⁵³
 - b. Easily accessible for patients, carers, families and consumers
- Standard 6 – Action 6.6 Correct identification and procedure matching
- The health service organisation specifies the:
 - a. Processes to correctly match patients to their care
 - b. Information that should be documented about the process of correctly matching patients to their intended care
- Standard 6 – Action 6.7 Clinical handover
- The health service organisation, in collaboration with clinicians, defines the:
 - a. Minimum information content to be communicated at clinical handover, based on best-practice guidelines
 - b. Risks relevant to the service context and the particular needs of patients, carers and families
 - c. Clinicians who are involved in the clinical handover
- Standard 6 – Action 6.11 Documentation of information
- The health service organisation has processes to contemporaneously document information in the healthcare record, including:
 - a. Critical information, alerts and risks
 - b. Reassessment processes and outcomes
 - c. Changes to the care plan
- Standard 8 – Action 8.4 Recognising acute deterioration
- The health service organisation has processes for clinicians to detect acute physiological deterioration that require clinicians to:
 - a. Document individualised vital sign monitoring plans
 - b. Monitor patients as required by their individualised monitoring plan
 - c. Graphically document and track changes in agreed observations to detect acute deterioration over time, as appropriate for the patient

12. References

1. ADS-ANZCA Perioperative Diabetes and Hyperglycaemia Guidelines November 2022

13. Policy owner (relating to these procedures)

Enquiries relating to this procedure may be directed to:

Title: Nursing Coordinator
Department: Perioperative
Email: shane.hastings@health.wa.gov.au

14. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V3	07/09/2023	07/09/2026	Addition of management of diabetic patients

15. Authorisation

This procedure has been authorised and issued by the Director Clinical Services.

Authorised by	Dr Alison Parr – Director Clinical Services
Authorisation date	05/09/2023
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